Rajasthan's State Health Insurance and Household Welfare

A Border-District Difference-in-Differences Study of the Bhamashah Swasthya Bima Yojana (BSBY)

Your Name

Course / Department, IIT Roorkee

November 2, 2026



Table of Contents

1. Motivation
2. Literature
3. Policy & Question
4. Data & Method
5. Results
6. Cost-effectiveness
7. Conclusions

Motivation

- Indian households pay a large share of health costs **out of pocket** (OOP); one hospitalisation can push a poor family into debt.
- Policy response: **tax-funded hospital insurance**. Government pays an insurer a premium per family; families get cashless care.
  - National: RSBY (2008) $\rightarrow$ PM-JAY (2018)
  - States: Karnataka, Maharashtra, Tamil Nadu, Gujarat, ...
  - **Rajasthan: BSBY, 13 December 2015**
- Evidence is **mixed**: RSBY did not reduce OOP; Karnataka's scheme cut OOP and mortality.
- **BSBY has not been evaluated against a credible comparison group.**

Literature (1): Dube, Lester & Reich (2010)

Review of Economics and Statistics 92(4)

Introduction & objective

National studies of US minimum wages found job losses; local case studies did not. Objective: estimate effects using only **counties that touch across a state border**.

Method

County data 1990–2006 (QCEW); $\sim$316 contiguous cross-border county pairs; county FE + **pair-specific time effects**; SEs clustered by state and border segment.

Results

Earnings elasticity 0.15–0.23 (all specs). Employment: national specs -0.18 to -0.21; contiguous pairs ≈ 0 (0.016). National estimates are biased by **spatial differences in trends**.

Our use: the border-pair design, pair fixed effects, clustering.

Literature (2): Karan, Yip & Mahal (2017)

Social Science & Medicine 181

Introduction & objective

RSBY (2008): Rs 30,000 hospital cover, ∼4 crore families. Earlier studies lacked a comparison group. Objective: causal effect of RSBY on OOP spending.

Method

NSS consumer expenditure rounds 1999–2000, 2004–05 (pre) and 2011–12 (post), ∼3.5 lakh households. Poorest two quintiles in RSBY vs non-RSBY districts, propensity-score matched. DiD with **two pre-periods** (ϕ_2 pre-trend, ϕ_3 effect). States with own schemes dropped.

Results

No effect on inpatient OOP, its level, or catastrophic spending. Probability of any OOP **rose ∼30%**. Non-medical spending +5%.

Our use: comparison group's scheme; OOP benchmark; ϕ_2/ϕ_3; RSBY premium (Rs 560); Rajasthan in early RSBY rollout (p. 84).

Literature (3): Sood et al. (2014)

BMJ 349, g5114

Introduction & objective

Karnataka's Vajpayee Arogyashree (2010): free tertiary care for below-poverty-line (BPL) households, rolled out first in northern districts. Objective: effect on OOP, hospital use and mortality.

Method

Geographic regression discontinuity: 572 villages on either side of the early/late rollout boundary, propensity-matched; ∼60,000 households surveyed (Sept 2012). Placebos: above-poverty-line households, uncovered conditions.

Results

Mortality from covered conditions 0.32% vs 0.90% (−64%); OOP for covered tertiary care −64%; utilisation +44% ($p = 0.06$). No effect for ineligible households.

Our use: closest in topic and method; placebo logic; village-level RD as future work.

Literature (4): other studies

Study	Method / finding	Our use
Card & Krueger (1994)	NJ vs PA DiD; no job loss	One state vs neighbour
Abadie et al. (2010)	Synthetic control; placebo states	Permutation inference
Conley & Taber (2011)	Few treated groups: SEs too small	Why permutation
Dupas & Jain (2021)	BSBY claims; women under-served	Same scheme, other question
Kling et al. (2007)	Standardised effect sizes	SD-unit column
Dell (2010)	Spatial regression discontinuity	Future work
Dhaliwal et al. (2013)	Cost per unit of causal impact	Cost-effectiveness
Finkelstein & McKnight (2008); Hendren & Sprung-Keyser (2020)	Value of risk protection; MVPF	Future welfare work

Gap: no study estimates BSBY's effect with a comparison group.

The policy: BSBY

Design (SIHFW Rajasthan deck)

- Launched 13 Dec 2015, ∼1 crore families
- NFSA + RSBY families, ∼**67% of population**
- Rs 30,000 secondary + Rs 3 lakh tertiary per family per year
- 1,715 packages; cashless; premium paid by state
- Claims paid: **Rs 295.63 cr** (13 Dec 2015 – 10 Feb 2017)

Timeline

2005–06	NFHS-3 (pre-trend)
2008	RSBY nationally
Dec 2015	**BSBY launched**
Jan–Jul 2016	NFHS-4 in Rajasthan
2018	PM-JAY nationally
2019	BSBY $\rightarrow$ AB-MGRSBY
2019–21	NFHS-5 (post)
2023–24	NFHS-6 (third round)

Research question and hypotheses

Question

Did BSBY raise health insurance coverage and reduce out-of-pocket spending, relative to districts just across the state border that had only the national schemes?

- **H1 (coverage):** BSBY raised the share of insured households.
- **H2 (financial protection):** BSBY reduced OOP spending.
- **H3 (use of care):** BSBY changed institutional / public-facility births.
- **Placebos:** no effect on electricity or women's schooling.

Estimand: Rajasthan's scheme *relative to* RSBY/PM-JAY, not insurance vs none.

Data

Source	Use
NFHS-4 (2015–16), NFHS-5 (2019–21) district factsheets	Main analysis (one value per district)
NFHS-3/4/5 state factsheets	Pre-trend event study
NFHS-6 (2023–24), all border districts, provisional	Third round; cross-checks
Census 2011 district map	Finding border pairs
SIHFW BSBY presentation	Claims, eligibility (cost)

Unit of observation: district (Census 2011 boundaries), one value per indicator per survey round. Variables and sample: next slide.

Variables and sample

NFHS indicator (exact wording)	Unit / treatment	Role	Rounds
Households with any usual member covered under a health insurance/financing scheme	% of households	**Main outcome (H1)**	NFHS-3*, 4, 5, 6
Average out-of-pocket expenditure per delivery in a public health facility	Rs, real 2019–21 (CPI-C); capped at $Q_3+1.5$IQR; also in logs	Welfare outcome (H2)	NFHS-4, 5 (dropped in NFHS-6)
Institutional births	% of births in last 5 years	Use of care (H3)	NFHS-4, 5, 6
Institutional births in public facility	% of births in last 5 years	Use of care (H3)	NFHS-4, 5, 6
Population living in households with electricity	% of population	Placebo	NFHS-4, 5, 6
Women with 10 or more years of schooling	% of women 15–49	Placebo	NFHS-4, 5, 6

* state level only (pre-trend test).

Treatment and time

- $D_i = 1$ if the district is in Rajasthan, 0 if a neighbour
- Pre = NFHS-4 (2015–16); Post = NFHS-5 (2019–21)
- Outcome in the regression: $\Delta Y = Y^{\text{NFHS-5}} - Y^{\text{NFHS-4}}$

Sample

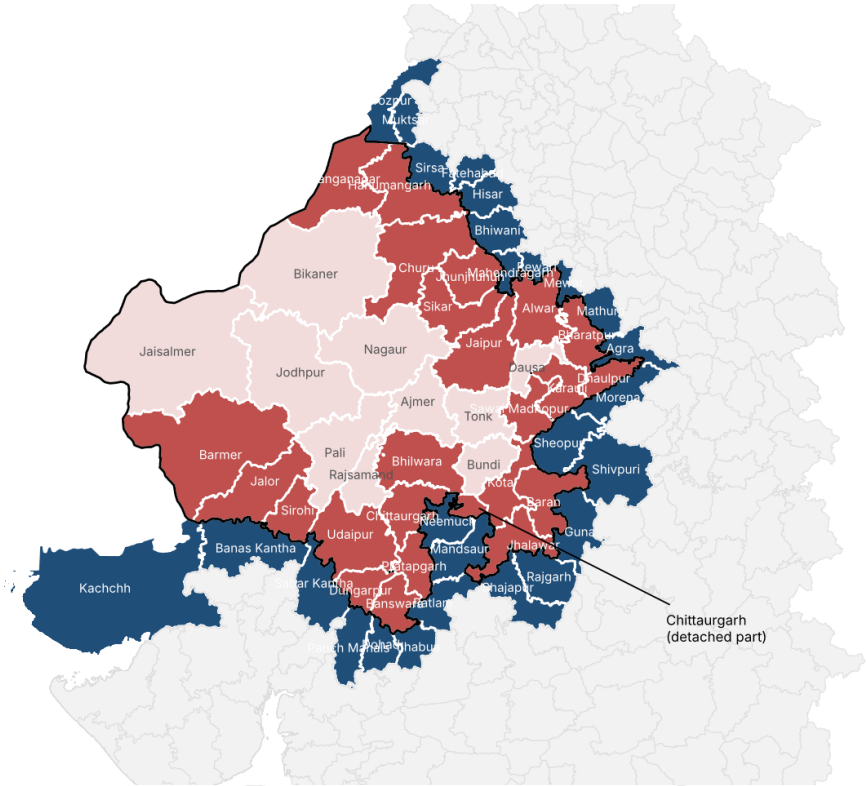
- 23 Rajasthan + 26 neighbour border districts; 51 pairs
- 5 neighbours split after NFHS-4 dropped
- **40 pairs**: 19 Rajasthan + 21 neighbour districts

Data cleaning (all checks pass)

Step	Rule / outcome
Missing values	No imputation; 0 = suppressed code $\rightarrow$ missing
Boundary changes	5 neighbour districts split after NFHS-4 dropped (51 $\rightarrow$ 40 pairs)
Cross-source	100% exact vs second copy; 33/33 vs NFHS-6 PDF
NFHS-6	Official compendiums, 6 states; 44 border districts found; 220/220 = independent compilation; Gujarat NFHS-5 insurance revision documented
Ranges & logic	% in [0,100]; public births $\leq$ institutional births: 0 violations
Inflation	OOP to real 2019–21 Rs (CPI-Combined)
Outliers	OOP capped at $Q_3 + 1.5\,\mathrm{IQR}$; **no border district reaches the cap**
Standardisation	z-scores with NFHS-4 neighbour mean and SD, both rounds
Baseline balance	Rajasthan higher OOP (+Rs 594, p = 0.09) $\Rightarrow$ baseline-control spec

Analysis scripts **refuse to run** unless the audit passes.

Border sample



23 Rajasthan × 26 neighbour districts

- Rajasthan border (treated)
- Neighbour border (control)
- Rajasthan interior (excluded)

- Districts **touching** across the state line (2011 map; ≥ 7 km shared border)
- 51 pairs: MP 22, Haryana 13, Gujarat 10, Punjab 3, UP 3
- **40 pairs** after dropping split districts
- Whole districts only

Empirical strategy

For district i in contiguous pair p:

$$\Delta Y_{ip} = \beta \cdot \mathit{Rajasthan}_i + \gamma_p + \varepsilon_{ip}, \qquad \Delta Y = Y^{\text{NFHS-5}} - Y^{\text{NFHS-4}}$$

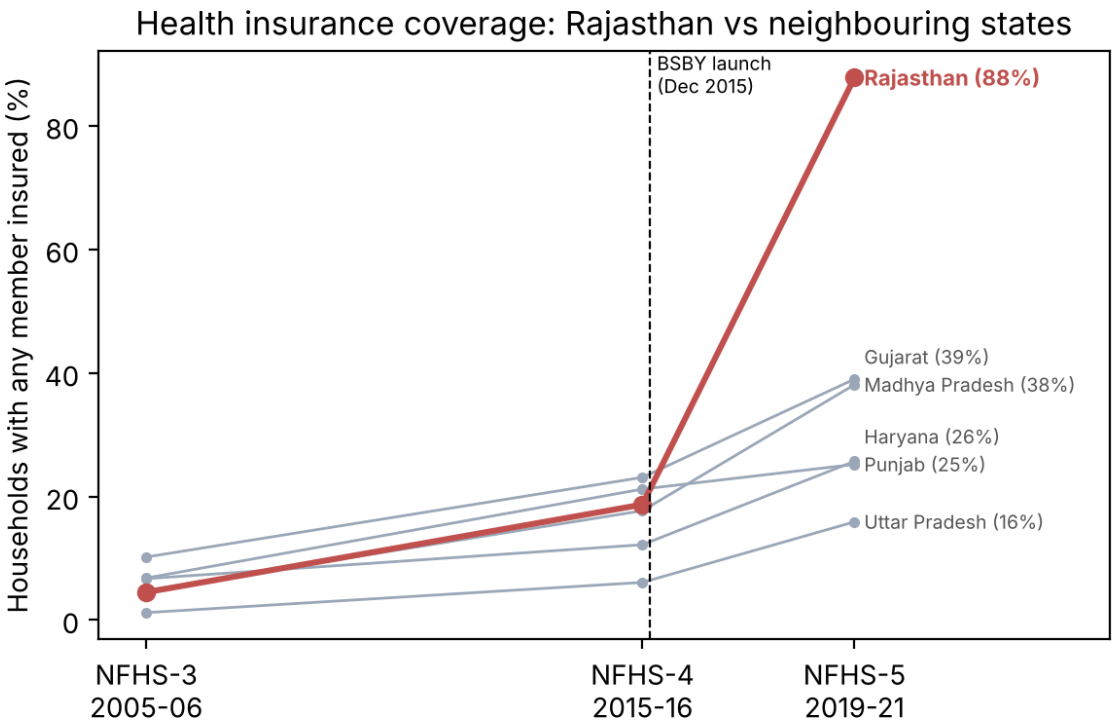
- ΔY: differencing removes everything fixed about a district (district FE)
- γ_p: pair fixed effect $\Rightarrow$ compare only with the touching neighbour
- β: within-pair difference-in-differences
- SEs **two-way clustered** by Rajasthan and neighbour district

Identifying assumption

Parallel trends across the border: without BSBY, both sides would have changed alike. Tested with a three-round state event study.

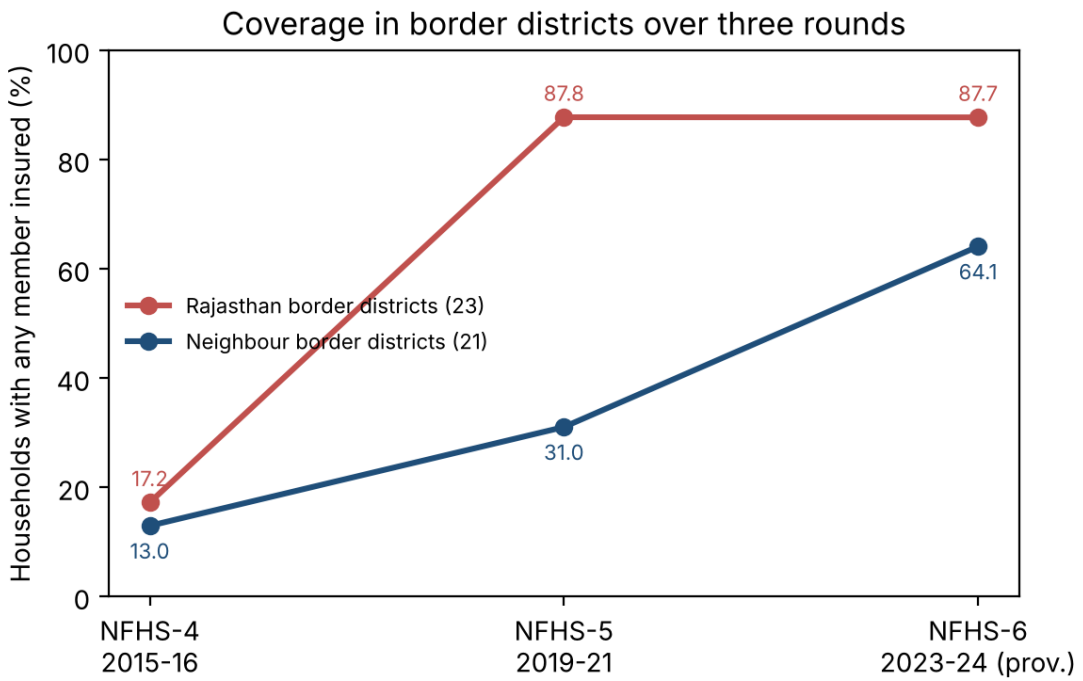
Robustness: drop Gujarat (own scheme); MP border only; pairs $\geq$ 10 km; control for NFHS-4 level; leave-one-out; OOP capping; placebos.

Pre-trends: state event study



- Pre-period DiD $\phi_2 = +4.1$ pp: roughly parallel
- Post-period DiD $\phi_3 = +56.4$ pp
- Trend-adjusted: **+52.3 pp**
- Permutation: Rajasthan **1st of 27** states, p = 0.037

Main result: coverage



NFHS-6 is provisional (official IIPS state compendiums). For 3 Gujarat districts the NFHS-6 compendium revises NFHS-5 insurance up ~5 pp.

- Rajasthan: 17.2 $\rightarrow$ 87.8 $\rightarrow$ 87.7%
- Neighbours: 13.0 $\rightarrow$ 31.0%
- Raw DiD: 70.6 $-$ 18.0 = 52.6
- **Regression: β = +52.8 pp (SE 5.1), p $<$ 0.001**
- Neighbours reach 64.1% by 2023–24 (next slide)

Did Rajasthan's lead last?

Insurance coverage in border districts (% of households)

	2015–16 NFHS-4	2019–21 NFHS-5	2023–24 NFHS-6
Rajasthan	17	**88**	**88**
Neighbours	13	31	**64**

- **2015–21:** BSBY puts Rajasthan 53 pp ahead
- **2021–24:** neighbours catch up under PM-JAY
- **2023–24:** Rajasthan still **15 pp** ahead (p = 0.03)

Takeaway

BSBY gave Rajasthan near-universal coverage **years before** the national scheme reached its neighbours.

NFHS-6 provisional (official IIPS compendiums). 15 pp = border-pair estimate excluding 3 Gujarat districts with revised NFHS-5 figures. Placebos flat 2019–24; women's schooling +3.6 pp over 2015–24 (p = 0.04).

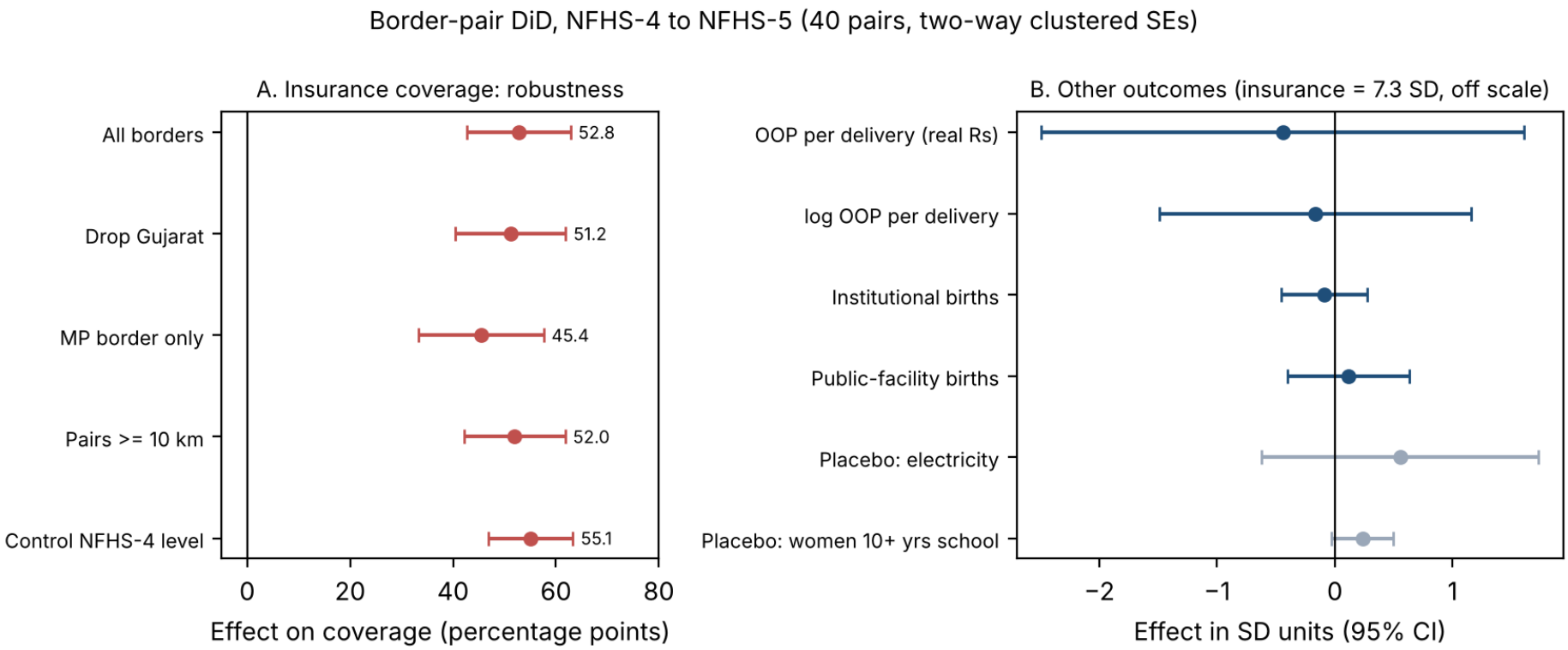
All outcomes

40 pairs, two-way clustered SEs

Outcome	β (SE)	p	SD units	+ NFHS-4 level
Insurance coverage (pp)	**+52.8 (5.1)**	**<0.001**	7.3	+55.1 (4.2)
OOP per delivery (real Rs)	−356 (840)	0.67	−0.44	+227 (212)
log OOP	−0.08 (0.31)	0.80	−0.17	+0.09 (0.12)
Institutional births (pp)	−1.2 (2.3)	0.62	−0.09	+0.8 (1.3)
Public-facility births (pp)	+2.4 (5.4)	0.66	0.12	+5.5 (3.9)
Placebo: electricity (pp)	+3.2 (3.5)	0.35	0.56	−0.1 (0.6)
Placebo: women 10+ yrs school	+3.1 (1.7)	0.08	0.24	+2.8 (1.5)

- Coverage: drop Gujarat +51.2; MP only +45.4; pairs $\geq$10 km +52.0; leave-one-district-out +51.3 to +53.9
- Nothing else significant in any sample; women's-schooling placebo borderline

Robustness at a glance



Panel A: coverage across specifications. Panel B: other outcomes in SD units (95% CI); grey = placebos.

Out-of-pocket spending

The estimated effect of BSBY on out-of-pocket spending per delivery is −Rs 356 and is not statistically significant. We interpret this as inconclusive rather than as evidence of no effect, for three reasons.

1. **Low precision.** The 95% confidence interval (−Rs 2,003 to +Rs 1,291) is wider than the baseline mean of Rs 2,235. The minimum detectable effect (Rs 2,352) exceeds the baseline, so even a complete elimination of delivery costs would probably go undetected.
2. **Unstable point estimate.** Rajasthan's border districts started with higher spending. Controlling for initial levels reverses the sign (+Rs 227), and excluding a single district (Baran) gives +Rs 30.
3. **Limited relevance of the outcome.** Deliveries in public facilities have been free under JSSK since 2011 in all states. BSBY mainly finances hospitalisations, for which NFHS reports no district-level spending.

These findings are consistent with Karan, Yip & Mahal (2017), who report no reduction in out-of-pocket spending under RSBY.

Cost-effectiveness: approach

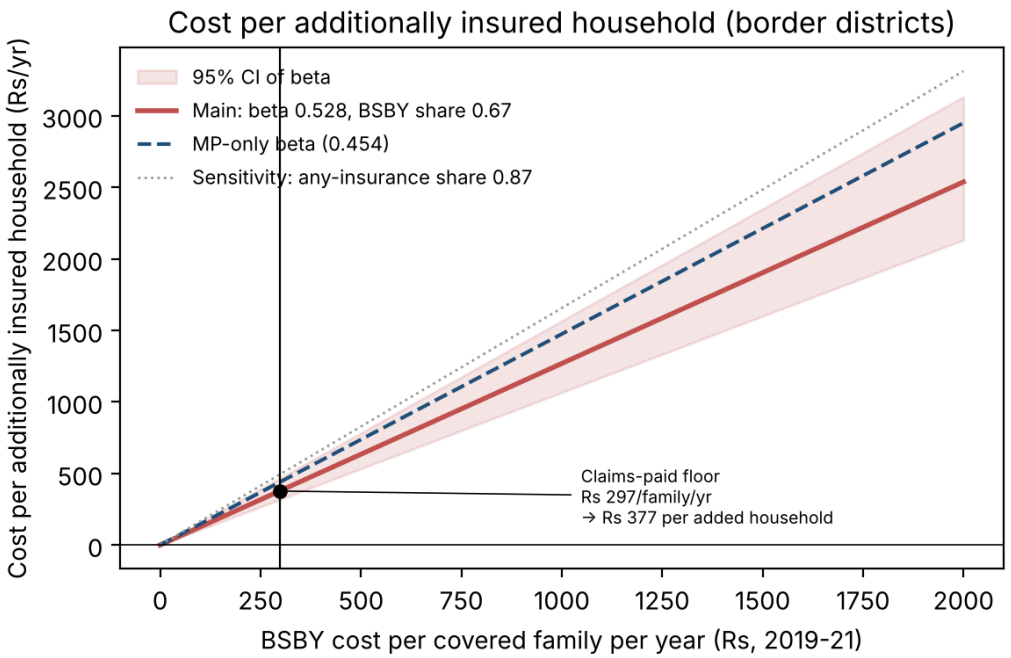
The benefits of BSBY cannot be valued in rupees, as the effect on out-of-pocket spending is inconclusive. We therefore report an incremental cost-effectiveness ratio (Dhaliwal et al. 2013): the annual public cost per household that became insured because of BSBY.

$$\text{Cost per additionally insured household} = \frac{c \cdot s - p_{RSBY} \cdot c_{cf}}{\beta}$$

	Definition	Value	Source
c	Annual cost per covered family (claims paid)	Rs 297	SIHFW, pp. 2, 19
s	Share of households covered by BSBY	0.67	SIHFW, p. 17
β	Increase in coverage caused by BSBY	0.528	Border-pair DiD
p_{RSBY}	Counterfactual cost per family under RSBY	0	Karan et al., p. 84

c: Rs 295.63 crore of claims over 1.16 years for about 1 crore families, i.e. Rs 254 per family per year (Rs 297 in 2019–21 prices). Claims understate the premium, so c is a lower bound. $p_{RSBY} = 0$ because RSBY was still being rolled out in Rajasthan in 2016, so the term $p_{RSBY} \cdot c_{cf}$ (with c_{cf} the coverage that would have existed without BSBY) drops out.

Cost-effectiveness: results



Cost per additionally insured household (Rs per year)

Value of β	Cost
Main (0.528)	**377**
95% CI (0.43–0.63)	317–466
MP border only (0.45)	439

BSBY cost at least Rs 377 a year for each household that became insured because of the scheme. About 21% of spending went to households that would have been insured anyway. For the scheme to break even, each additionally insured household must gain at least Rs 377 a year.

The estimate refers to the 2019–21 coverage gap; as neighbouring districts caught up, the cost per unit of lasting advantage rises.

Discussion

- **Coverage:** border estimate (+52.8) and trend-adjusted state estimate (+52.3) agree; by 2023–24 PM-JAY narrowed the gap to $\sim$15 pp, so BSBY's gain was mainly *timing*.
- **Financial protection:** our OOP null agrees with RSBY evidence (Karan et al.) and does not contradict Karnataka's tertiary-care gains (Sood et al.): we measure a margin insurance barely touches, at district resolution.
- **Who benefits:** Dupas & Jain find women use BSBY less; household coverage cannot see this.
- **Value for money:** $\geq$ Rs 377 per additionally insured household; $\sim$21% of spending reaches households that would have been insured anyway.

Conclusions

1. **BSBY worked as a coverage policy, and worked early:** +53 pp relative to the border by 2019–21, robust to every check; neighbours caught up to within 15 pp by 2023–24.
2. **Financial protection is unproven, not disproven:** OOP per delivery is the wrong margin and the design is underpowered for it.
3. **Cost:** at least Rs 377 per additionally insured household per year (against the 2019–21 gap); benefits must exceed this to justify it.
4. **Policy:** a state scheme can deliver near-universal coverage years before the national scheme does; evaluations should measure hospitalisation spending and health, not just enrolment.

Limitations and future work

Limitations

- NFHS-4 baseline began 6 weeks after launch $\Rightarrow \beta$ is a lower bound
- Scheme vs scheme, not insurance vs none
- District averages; large districts
- OOP: deliveries only, underpowered
- One pre-period, state level
- Cost is a claims floor; benefits not monetised
- NFHS-6 provisional; Gujarat NFHS-5 insurance revised
- Long-difference schooling placebo significant (p = 0.04)

Future work

- NSS 71st vs 75th: OOP per hospitalisation
- NFHS GPS clusters: spatial RD (Dell 2010; Sood et al. 2014)
- Link neighbours' catch-up to PM-JAY rollout
- Actual premium $\Rightarrow$ MVPF / value of risk protection

References

- Abadie, A., Diamond, A., & Hainmueller, J. (2010). Synthetic control methods for comparative case studies. *JASA*, 105(490), 493–505.
- Card, D., & Krueger, A. B. (1994). Minimum wages and employment. *American Economic Review*, 84(4), 772–793.
- Conley, T. G., & Taber, C. R. (2011). Inference with “difference in differences” with a small number of policy changes. *REStat*, 93(1), 113–125.
- Dell, M. (2010). The persistent effects of Peru’s mining mita. *Econometrica*, 78(6), 1863–1903.
- Dhaliwal, I., Duflo, E., Glennerster, R., & Tulloch, C. (2013). Comparative cost-effectiveness analysis to inform policy in developing countries. J-PAL.
- Dube, A., Lester, T. W., & Reich, M. (2010). Minimum wage effects across state borders. *REStat*, 92(4), 945–964.
- Dupas, P., & Jain, R. (2021, rev. 2023). Women left behind: Gender disparities in utilization of government health insurance in India. NBER WP 28972.
- Finkelstein, A., & McKnight, R. (2008). What did Medicare do? *Journal of Public Economics*, 92(7), 1644–1668.
- Hendren, N., & Sprung-Keyser, B. (2020). A unified welfare analysis of government policies. *QJE*, 135(3), 1209–1318.
- IIPS. National Family Health Survey (NFHS-3, 4, 5) fact sheets; NFHS-6 state compendiums: Rajasthan, Gujarat, Haryana, MP, Punjab, UP (provisional, 2026).
- Karan, A., Yip, W., & Mahal, A. (2017). Extending health insurance to the poor in India: RSBY and out of pocket spending. *Social Science & Medicine*, 181, 83–92.
- Kling, J. R., Liebman, J. B., & Katz, L. F. (2007). Experimental analysis of neighborhood effects. *Econometrica*, 75(1), 83–119.
- Patra, N. K. (2026). NFHS-6 provisional district-level data (India): independent compilation. GitHub (cross-check).
- SIHFW Rajasthan. *Bhamashah Swasthya Bima Yojana* (presentation). Jaipur.
- Sood, N., et al. (2014). Government health insurance for people below poverty line in India. *BMJ*, 349, g5114.

Thank you.

Code, data and full explainer:
`https://github.com/ipsanmoljj/india-welfare-analysis`